

QUALITY ASSURANCE SURVEILLANCE PLAN

For: Social Work Services/Health Care for Homeless Veterans Program

Contract Number:

Contract Description: The contractor shall provide 11 Emergency Residential Contract beds for Homeless Veterans. The contractor will also provide therapeutic and supportive services, room and board, and laundry facilities.

Contractor's name: Salvation Army (hereafter referred to as the contractor).

1. PURPOSE

To specify the systematic methods for evaluating performance of the Emergency Contract Housing program.

A. Scope:

Residential Contract Housing program will provide immediate housing to eligible homeless Veterans. They will provide the following:

ROOM AND BOARD: Room and Board to include a bed and other furnishings such as a dresser, storage, and personal linens (towels and bed sheets). Meals to include three (3) nutritionally adequate meals a day, 7 days a week and availability of nutritious snacks between meals and bedtime for those requiring or desiring additional food, when it is not medically contraindicated.

LAUNDRY FACILITIES: Laundry facilities for residents to do their own laundry or to have laundry done free of charge.

THERAPEUTIC AND SUPPORTIVE SERVICES: Therapeutic and Supportive Services determined to be needed by each individual Veteran referred for treatment as stated in the plan developed by the contractor, with input from the Veteran and the VA Homeless Program Coordinator (or designee). Services which the contractor shall furnish:

1. Will accept eligible Veterans on same day they present needing shelter.
2. Structured group activities as appropriate – examples include group therapy, social skills training, Alcoholics Anonymous, Narcotics Anonymous, vocational counseling and physical activities as appropriate.
3. Collaboration with the VA program staff, which shall provide supportive psychosocial services.

4. Individual professional counseling, including counseling on self-care skills, adaptive coping skills and as appropriate, vocational rehabilitation counseling, in collaboration with VA program and community resources.
5. Assistance to develop responsible living patterns, to maintain an acceptable level of personal hygiene and grooming, and to achieve a more adaptive level of psychosocial functioning, upgraded social skills, and improved personal relationships.
6. Provide close monitoring of medications. In addition to assuring the count and actual consumption, the Contractor shall assure that medication expiration date are tracked. The Contractor shall assure timely access to as needed (PRN) medications.
7. Skilled professional counselor that is licensed by appropriate licensing/regulatory boards to provide individual therapy and/or counseling to address mental health concerns including substance abuse/dependence. The Counselor(s) possess assessment and crisis intervention skills.
8. Assistance to gain and to apply knowledge of the illness/recovery process.
9. A drug and alcohol free setting and consistent support for drug and alcohol free lifestyle. The contractor shall have a system to do admission and random breathalyzers as needed. The Veterans Affairs Medical Center (VAMC) will provide drug screening upon request, in a timely manner.
10. A program that promotes community interaction.

In addition, the Contractor shall collaborate with the VA program staff, who will conduct treatment and discharge planning reflecting a team assessment of health, psychosocial and vocational needs and the involvement of resident' families and appropriate community resources in resolving problems and setting goals.

The Contractor shall have the right to discharge beneficiaries to preserve the health and safety of other beneficiaries and staff. The HCHV Coordinator or designee shall be immediately notified of such discharge by the next business day. Other routine

discharges due to rule violations must be approved through the HCHV Coordinator or designee. In emergent situations, the Contractor shall evaluate and link Veterans to an appropriate level of care management.

In the event of a medical emergency, the Contractor is responsible for transporting the Veteran to the VA Medical Center, unless the Health Care for Homeless Veterans (HCHV) Coordinator or designee determines an ambulance transportation is needed.

The Contractor shall maintain an individual record on each Veteran placed in the facility by the HCHV Program. The Contractor must comply with the requirements of the Confidentiality of Medical Records (38 USC 4132) and the Confidentiality of Alcohol and Drug use of a Veteran (42 CFR part II) when appropriate, and is considered a part of this contract. All case records shall be made available on a need to know basis to appropriate VAMC staff members involved with the treatment program of the Veteran concerned. In addition to reasons for referral, the clinical record, maintained by the contract facility shall include the following:

1. All essential identifying data relevant to the resident. Records should reflect admission and discharge summaries, weekly progress notes and interim notes documenting any significant alterations in functioning.
2. Data relating to the resident's admission, to include the targeted goals for constructive changes, and the anticipated length of stay.
3. Reports of periodic re-evaluation by program staff, to include any measures of movement toward rehabilitation goals, with particular focus on the attainment of self-help skills. Treatment plans will be reviewed monthly.
4. Develop a budget plan and/or a vocational plan in preparation for moving into Permanent Housing.
5. Final summaries on each Veteran who leaves the program, to include a description of beneficial changes realized during the residential period, reasons for leaving, the resident's future plans and follow-up locator information.
6. Upon discharge or death of the Veteran, medical records on all Veterans will be retained by the residential treatment facility for a period of at least three (3) years following termination of care.
7. USC 4132/42 CFR Part II is available for review at the Veterans Affairs Medical Administration Service.

The contractor shall collaborate with the VA program staff, who shall conduct treatment and discharge planning reflecting a team assessment of health, psychosocial and vocational needs and the involvement of residents' families and appropriate community resources in resolving problems and setting goals. Routine discharges due to rule violations must be approved through the HCHV Coordinator or designee.

The HCHV Coordinator or case manager shall identify and refer all patients to the Contractor. All patients will have co-occurring mental health and/or substance use/dependency problems.

Veteran's dietary needs shall be met in accordance with sound medical practices.

1. At least three (3) nutritious meals or the equivalent shall be served daily at regular times with not more than fourteen (14) hour span between the evening and breakfast of the following day. Between meals and bedtime snacks of nourishing quality shall be offered.
2. There shall be evidence that food is prepared, served and stored under sanitary conditions.
3. The sanitary procedures shall be established and maintained for washing dishes, cleaning equipment, work areas and proper waste disposal.
4. The Contractor shall comply with applicable requirement of the Confidentiality of Alcohol and Drug Abuse Patient Records (42 CFR Part II) and the Confidentiality of Certain Medical Records (38 USC 7332).

All patients shall be capable of self-preservation, and in emergency situations, will have sufficient capability to recognize physical dangers, sufficient judgment to recognize when danger requires immediate egress from the group residence, and sufficient physical mobility to accomplish such egress.

The Department of Veterans Affairs Medical Center reserves the right to inspect the residential treatment facility and all appurtenances by an authorized representative(s) designated by the Alexandria VAMC

Before award and on an annual basis thereafter, the residential treatment community healthcare center/halfway house will be inspected by a multidisciplinary Alexandria VAMC inspection team and the Engineer Fire/Life Safety Officer. The inspection shall include but is not limited to the following areas:

1. General observation of residents indicated that they maintain an acceptable level of personal hygiene and grooming.
2. The facility meets applicable fire, safety and sanitation standards maintained in attractive surroundings conducive to social interaction and the fullest development of the resident and of the resident's rehabilitative potential.
3. The facility should be in a central location near public transportation and near areas that provide employment.
4. There is evidence of facility-community interaction. This may be demonstrated by the nature of scheduled activities or information about resident flow out of the facility e.g. community activities, volunteers, local consumer service, etc.
5. Staff behavior and interaction with the residents convey an attitude of genuine concern and caring.
6. Appetizing, nutritional adequate meals are provided. The local VAMC dietitian may consult with the inspection team and the team making subsequent assessments, in evaluating not only the printed menu(s), but also the patient's satisfaction with meals and the actual consumption of the food offered.

In the event a Veteran receiving care under this contract dies, the residential treatment facility shall promptly notify the HCHV Coordinator or designee and immediately assemble, inventory, and safeguard the Veteran's personal effects. The funds, deposits, and effects left by Veteran upon the premises of the residential treatment facility shall be delivered by the staff of the facility to the person or persons entitled thereto under the laws currently governing the facility for making disposition of funds and effects left by the Veteran, unless he died without leaving a will, heirs or next of kin capable of inheriting. When disposition has been made, the itemized inventory with a notation as to the disposition of the funds and effects will be immediately forwarded to the HCHV Coordinator or designee. Should a deceased Veteran have no will, heirs, or next of kin, his/her personal property and funds wherever located vests in and becomes the property of the United States in trust. In these cases the residential treatment facility shall forward inventory of any such property and funds in its possession to the VAMC, and will then (except articles of clothing necessary for proper burial) under safeguard until instructions are received from the VA concerning disposition.

The Contractor shall comply with the principles listed in 38 CFR 17.707(b) to provide housing and supportive services in a manner that is free from religious discrimination. **SUPPLEMENTAL SERVICES.** Unless specifically excluded in this contract, the per diem rate established will include the services listed in this document and will also include all services or supplies normally provided other patients by the facility without incurred cost to the Government. It is impossible to determine the exact or estimated amount which will be incurred. Therefore, no obligation will be incurred by the Department of Veterans Affairs under this contract unless a specific authorization for Veterans is issued by the VAMC for services rendered in accordance with such authorization upon receipt of proper invoices by the residential treatment center at the completion of each month's service.

ADDITIONAL CONTRACT REQUIREMENTS

PERSONNEL

The contractor shall employ sufficient qualified personnel that by education and training (and, when required, certification or licensure) to provide the Basic Services and Supplemental Services required by this Statement of Work. There will be at a minimum, a full time administrative staff member or a designated employee on duty on the premises, or residing at the program and available for emergencies, 24 hours a day, 7 days a week. In those instances where a supervised residential setting is linked to a geographically distinct rehabilitation and/or socialization program, sufficient professional health care personnel shall be identified to provide the necessary therapeutic activities and to ensure a meaningful integration of these efforts with those provided in a residential setting.

The Contractor shall identify each person functioning as "Key Personnel" under this contract, and provide to the VA a description of the services to be provided by such person, together with a resume summarizing that person's relevant skills and experience. Key personnel shall include a Program Director, Clinical Director, cook and a House Manager. The Contractor shall notify the HCHV Coordinator or designee of all changes in staff.

All Contractor staff providing services to Veterans under this contract shall undergo the appropriate background investigation and will provide a copy of the result to the HCHV Program Coordinator or designee for review. Before receiving an initial referral of Veterans under the contract, the Contractor shall submit a listing of all Contractor personnel proposed to provide services to Veterans under the contract.

B. REFERRALS

The VA is responsible for determining eligibility of Veterans prior to referral to the Contractor for treatment. The HCHV Coordinator or case manager shall identify and refer all patients to the Contractor. A list of authorized VA ordering officials for the contract shall be provided to the Contractor upon award of the contract. Ordering officials may be added or deleted from the list during the term of the contract at the discretion of VA Contracting Officer. The Contractor shall be provided an updated list of authorized VA ordering officials whenever such changes are made.

It is understood that the Contractor shall not be paid for care provided to a referred Veteran beyond the period authorized in the referral, unless an extension of the authorization is provided verbally or in writing by the HCHV Coordinator or designee.

NOTE: THE CONTRACTOR SHALL REFER FEMALE VETERANS IN NEED OF SERVICES TO THE HCHV COORDINATOR OR DESIGNEE.

C. ABSENCES AND CANCELLATION

Should a Veteran require hospitalization, payment shall be continued for a maximum period of two days (48 hours). Upon the Veterans' discharge from the hospital shall be granted priority placement at a contract residential facility if a bed is available and Veteran is deemed appropriate for contract housing.

The contractor shall notify the VA of unauthorized absences by a referred Veteran from the facility. Should a Veteran absent himself from the Contractor's facility in an unauthorized manner, payment for services for that Veteran shall be continued for a maximum period of two days (48 hours), provided there is an active outreach attempt on the part of the Contractor's staff to return the Veteran to the facility and there is a reasonable belief that the Veteran will return. Absence, when discovered, shall be reported to the HCHV Coordinator or designee ASAP or by the close of the next business day. Extraordinary circumstance (e.g. absent Veteran is potentially suicidal) shall be made to the HCHV Coordinator or designee immediately. Reports shall be made to the HCHV Coordinator or designee during regular business hours and the Administration Officer during non-regular business hours.

VA reserves the right to remove any or all Veterans from the facility at any time, without additional cost, when it is determined to be in the best interest of the VA or the Veteran.

The contractor shall notify the authorizing VA facility immediately when a medical emergency occurs that requires hospitalization of a referred Veteran. It is agreed that the Veteran will be admitted to the appropriate VA facility. When such admission is not feasible because of the nature of the emergency, it is agreed that hospitalization in a non-federal hospital may be accomplished provided that VA authorization is obtained. If hospitalization of emergency, it is agreed that hospitalization in a non-federal hospital may be accomplished provided that VA authorization is obtained. If hospitalization of a non-emergency nature is required it is agreed that admission to the appropriate VA facility will be accomplished promptly.

The contractor shall notify the authorizing VA facility immediately of any incidents involving Veterans residing in the residential program. The contractor shall notify the HCHV Coordinator or designee by telephone during the hours of 8:am and 4:30pm. For all incidents that occur after normal business hours, the contractor should notify the Administrator on Duty (AOD). The contractor shall provide the HCHV Coordinator or designee with a copy of the incident report within 24 hours. The contractor shall maintain a copy of the incident report in the Veteran's case record.

It is understood that the Contractor will not be paid for care provided to a referred Veteran beyond the period authorized in the referral, unless an extension of the authorization is provided verbally or in writing by the HCHV Coordinator.

The length of stay should not exceed six (6) months. Each Veteran will be evaluated monthly on need and progress to determine the actual length of stay required. An extended length of stay is possible based upon circumstances and availability of funding. The decision will be made by the HCHV Coordinator or designee and the director of facility.

CONTRACTOR STAFF CONDUCT/COMPLAINTS HANDLING

Contractor personnel shall treat referred Veterans with dignity and respect and abide by standards of conduct mirroring those prescribed by current federal personnel regulations. The Contractor shall comply with the VA Patient's Bill of Rights as set forth in 38 CFR 17.34a (copy available upon request).

The VA reserves the right to exclude Contractor staff members from providing services to Veterans under this contract based on breaches of conduct, including conduct that jeopardizes patient care or interferes with the regular and ordinary operation of the facility: Breaches of conduct include intoxication or debilitation resulting from drug use, theft, patient abuse, dereliction or negligence in performing directed tasks, or other conduct resulting in formal complaints by Veterans or other staff members to designated Government representatives. The Contractor and HCHV Coordinator shall deal with issues raised concerning contractor personnel conduct. The HCHV Coordinator shall be the final arbiter on questions of acceptability and in validating complaints.

D. TRANSPORTATION

The Contractor shall assist the Veterans with local transportation to scheduled meetings and appointments. The Contractor shall be expected to help the Veteran access public transportation, including providing information and instructions necessary to enable Veterans to utilize public transportation. If adequate public transportation is not available or appropriate for a Veteran, the Contractor shall arrange for alternative transportation.

E. FACILITY

It is the responsibility of the Contractor to properly maintain its facilities and the VA shall have no responsibility for paying or reimbursing the Contractor for such expenses. The contract facility shall:

- Have a current occupancy permit issued by the local and state governments in the jurisdiction where the facility is located.
- Be in compliance with existing standards of State safety codes and local, and/or State health and sanitation codes.
- Where applicable, be accredited by the State.
- Be equipped with operational air conditioning /heating systems.
- Be kept clean free of dirt, grime, mold, or other hazardous substances and damaged noticeably detract from the overall appearance.
- Be equipped with first aid equipment and an evacuation plan in case of emergency.

- Have windows and doors that can be opened and closed in accordance with manufacturer standards.
- ▶ The contractor facility shall meet fire safety requirements, as follows:
 - The building shall meet the requirements of the applicable residential occupancy chapters of the current version of NFPA 101, National Fire Protection Association's Life Safety Code. Any equivalencies or variances shall be approved by the appropriate Veterans Health Administration Veterans Integrated Service Network (VISN) Director.
 - All residents in the facility shall be mentally and physically capable of leaving the building, unaided, in the event of an emergency.
 - Fire exit drills shall be held at least quarterly. Residents shall be instructed in evacuation procedures when the primary and/or secondary exits are blocked. A written fire plan for evacuation in the event of fire shall be developed and reviewed annually. The plan shall outline the duties, responsibilities and actions to be taken by the staff and residents in the event of a fire emergency. This plan shall be implemented during fire exit drills.
 - A written policy regarding tobacco smoking in the facility shall be established and enforced.
 - Portable fire extinguishers shall be installed at the facility. Use NFPA 10, Portable Fire Extinguishers, as guidance in selection and location requirements of extinguishers.
 - Requirements for fire protection equipment and systems shall be in accordance with NFPA 101. All fire protection systems and equipment, such as the fire alarm system, smoke detectors, and portable extinguishers, shall be inspected, tested and maintained in accordance with the applicable NFPA fire codes and the results documented.
 - The annual inspection by a VA team required by paragraph 7 shall include a fire and safety inspection conducted at the facility unless a review of past Department of Veterans Affairs inspections or inspections made by the local authorities indicates that a fire and safety inspection would not be necessary, in which case the fire and safety inspection may be waived by the VA.

G. INSPECTION OF FACILITY AND PROGRAM

Prior to the award of a contract and annually during the contract term, a multidisciplinary VA team consisting of a social worker, dietitian, registered nurse, a representative of the VA Police, and a Facilities Management Safety Officer, and other subject matter experts as determined necessary by the medical center director or HCHV Coordinator shall

conduct a survey of the Contractor's facilities to be used to provide Veterans food preparation, food storage, laundry facilities, restrooms, shelter, therapeutic services and all areas of the property that is assessable for the Veteran or that could directly impact the Veteran to assure the facility provides quality care in a safe environment. Inspections may also be carried out at such other times as deemed necessary by the Department of Veterans Affairs.

Special attention will be directed to the adequacy of the Veteran's records. Site visits will also include a spot check of records to ensure contractor invoices accurately reflect the Veteran's length of stay.

The contractor will be advised of the findings of the inspection team. If deficiencies are noted during any inspection, the contractor will be given a reasonable time to take corrective action and to notify the Contracting Officer that the corrections have been made. A contract will not be awarded until noted deficiencies have been eliminated. Failure by the Contractor to take corrective action within a reasonable time will be reported to the VA Contracting Officer. If corrections are not made to the satisfaction of the VA, the Contracting Officer will consult with the appropriate officials so that suitable arrangements can be made to discontinue plans to award a contract, or to discharge or transfer patients and to terminate the existing contract, as appropriate.

The inspection of the Contractor facilities will include inspection for conformity to the current Life Safety Code as described in paragraph 6, and will also include the following:

- General observation of residents to determine if they maintain an acceptable level of personal hygiene and grooming.
- Assessment of whether the facility meets applicable fire, safety and sanitation standards.
- Determining whether the facility is in attractive surroundings conducive to social interaction and the fullest development of the resident's potential.
- Observation of facility operations to see if appropriate organized activity programs are available during waking hours (including evenings) and degree to which a high level of activity is observed in the facility, such as individual professional counseling, physical activities, assistance with health and personal hygiene.
- Seeking evidence of facility-community interaction, demonstrated by the nature of scheduled activities or by information about resident flow out of the facility, e.g., community activities, volunteers, local consumer services, etc.
- Observation of staff behavior and interaction with residents to determine if they convey an attitude of genuine concern and caring.
- Inspecting the types of meals and other nutrition provided to residents to see if appetizing, nutritionally adequate meals are provided in a setting, which encourages social interaction and if nutritious snacks between meals and

bedtime are available for those requiring or desiring additional food, when it is not medically contraindicated.

- Making a spot check of Veterans' records to ensure accuracy with respect to Veterans' length of stay and services provided to the Veterans.
- All Department of Veterans Affairs reports of inspection of residential facilities furnishing treatment and supportive services to eligible Veterans shall, to the extent possible, be made available to all government agencies charged with the responsibility of licensing or otherwise regulating or inspecting such institutions.

H. CONTRACT CHANGES/TECHNICAL DIRECTIONS.

The Contracting Officer is the only person authorized to approve changes or modify any of the requirements of this contract. The Contractor shall communicate with the Contracting Officer on all matters pertaining to contract administration. Only the Contracting Officer is authorized to make commitments or issue changes that shall affect price, quantity or quality of performance of this contract.

IN THE EVENT THE CONTRACTOR AFFECTS ANY SUCH CHANGE AT THE DIRECTION OF ANY PERSON OTHER THAN THE CONTRACTING OFFICER WITHOUT AUTHORITY, NO ADJUSTMENT SHALL BE MADE IN THE CONTRACT PRICE TO COVER AN INCREASE IN COSTS INCURRED AS A RESULT THEREOF.

The COR will be responsible for the overall technical administration of this contract as outlined in the COR Delegation of Authority, including monitoring of the contractor's performance.

The contractor, their personnel, and their subcontractors shall be subject to the Federal laws, regulations, standards, applicable CARF standards, and VA Directives and Handbooks regarding information and information system security as delineated in this contract.

The C&A requirement do not apply to this contract and a Security Accreditation Package is not required.

Monitoring of Facility and Program:

Prior to the award of a contract and annually during the contract term, a multidisciplinary VA team consisting of a social worker, dietitian, registered nurse, a representative of the VA Police, and a Facilities Management Safety Officer, and other subject matter experts as determined necessary by the medical center director or HCHV Coordinator shall conduct a survey of the Contractor's facilities to be used to provide Veterans food preparation, food storage, laundry facilities, restrooms, shelter, therapeutic services and all areas of the property that is assessible for the Veteran or that could directly impact the Veteran to assure the facility provides quality care in a safe environment. Inspections may also be carried out at such other times as deemed necessary by the Department of Veterans Affairs.

Special attention will be directed to the adequacy of the veteran's records. Site visits will also include a spot check of records to ensure contractor invoices accurately reflect the veteran's length of stay.

Monitoring of the facility and program will be documented on VISN designed templates.

2. GOVERNMENT ROLES AND RESPONSIBILITIES

The following personnel shall oversee and coordinate surveillance activities.

a. Contracting Officer (CO) - The CO shall ensure performance of all necessary actions for effective contracting, ensure compliance with the contract terms, and shall safeguard the interests of the United States in the contractual relationship. The CO shall also assure that the contractor receives impartial, fair, and equitable treatment under this contract. The CO is ultimately responsible for the final determination of the adequacy of the contractor's performance.

Assigned CO: Thomas Kovarovic

Organization or Agency: Department of Veterans Affairs, Network Contracting Office 16,
Alexandria VA Health Care System

b. Contracting Officer's Representative (COR) - The COR is responsible for technical administration of the contract and shall assure proper Government surveillance of the contractor's performance. The COR shall keep a quality assurance file. The COR is not empowered to make any contractual commitments or to authorize any contractual changes on the Government's behalf.

Assigned COR: Paul Scheppf, LCSW

c. Other Key Government Personnel – Other HCHV staff

3. CONTRACTOR REPRESENTATIVES

The following employees of the contractor serve as the contractor's program manager for this contract.

a. Program Manager –

b. Other Contractor Personnel -

4. PERFORMANCE STANDARDS

Performance standards define desired services. The Government performs surveillance to determine if the contractor exceeds, meets or does not meet these standards.

The Performance Requirements Summary Matrix, paragraph G in the Performance Work Statement (PWS), includes performance standards. The Government shall use these standards to determine contractor performance and shall compare contractor performance to the Acceptable Quality Level (AQL).

Task	ID	Indicator	Standard	Acceptable Quality Level	Method of Surveillance	Incentive
Basic Services	C.a	Room and board to include a bed and other furnishings such as a dresser, storage and personal linens. Meals to include 3 nutritionally adequate meals daily, 7 days a week and availability of nutritious snacks between meals and bedtime for those requiring or desiring additional food, when not medically contraindicated.	Daily.	100%	Observation and random inspection.	Exercise of Option Periods
	C.b	Availability of laundry facilities for residents to do their own laundry or to have.	Daily	100%	<Observation and random inspection.	Exercise of Option Periods
	C.c	Therapeutic and Supportive Services individualized as stated in the plan developed by the contractor, with input from the Veteran and VA Homeless Program Coordinator or designee.	Daily	100%	Observation, random inspection and progress review.	<Exercise of Option Periods

Task	ID	Indicator	Standard	Acceptable Quality Level	Method of Surveillance	Incentive
	C.c 7	Skilled professional counselor that is licensed by appropriate licensing/regulatory boards to provide individual therapy and/or counseling to address mental health concerns including substance abuse/dependence. The Counselor(s) possess assessment and crisis intervention skills.	Daily.	100%	Observation and random inspection.	Exercise of Option Periods
	C.c 9	Drug and alcohol free setting and consistent support for drug and alcohol free lifestyle. To include random breathalyzers as needed.	Daily	100%	Observation and random inspection.	Exercise of Option Periods

Task	ID	Indicator	Standard	Acceptable Quality Level	Method of Surveillance	Incentive
	C.c 10	Individual Records must be maintained on each Veteran. To include essential identifying data relevant to the resident (weekly progress notes and interim notes documenting any significant alteration in functioning and treatment plan with identified problems and goals for achievement.)	Upon admission and throughout the Veteran's residency. Including discharge.	100%	Observation, random inspection and progress review.	Exercise of Option Periods
	Pg 9. 2.	Evidence that food is prepared, served and stored under sanitary conditions.	Daily	100%	Observation and random inspection.	Exercise of Option Periods.
	C.c 6	Provide Close monitoring of medication.	Daily	100%	Observation and random inspection.	Exercise of Option Periods.
Safety	Pg. 8	Confidentiality of all patient information.	Daily	100%	Observation and random inspection.	Exercise of Option Periods.
	Pg. 10	Provide housing and supportive services free from religious discrimination.	daily	100%	Observation and random inspection.	Exercise of Option Periods.
	E. Pg 13	Compliant with the NFPA life safety code.	At all times	100%	Observation and random inspection.	Exercise of Option Periods.

Task	ID	Indicator	Standard	Acceptable Quality Level	Method of Surveillance	Incentive
	E	Compliant with the American with Disabilities Act.	At all times	100%	Observation and random inspection.	Exercise of Option Period and past performance and/or any other incentive that was proposed in your solution.>
	13	Provide each Veteran appropriate space and security for themselves and their belonging.	At all times	100%	Observation and random inspection.	Exercise of Option Periods.
	Pg. 9	Provide sufficient sanitary facilities to resident that are in proper operational condition, may be used in privacy, and are adequate personal cleanliness and disposal of human waste. Provides adequate heating and or cooling of facility and electrical sources for use.	At all times	100%	Observation and random inspection.	Exercise of Option Periods.

5. INCENTIVES

The Government shall use Exercise of Option Periods as incentives.

6. METHODS OF QA SURVEILLANCE

Various methods exist to monitor performance. The COR shall use the surveillance methods listed below in the administration of this QASP.

Direct observation, periodic inspections, and progress meetings.

a. DIRECT OBSERVATION. (Can be performed periodically or through 100% surveillance.)

1-14

b. PERIODIC INSPECTION. (Evaluates outcomes on a periodic basis. Inspections may be scheduled [Daily, Weekly, Monthly, Quarterly, or annually] or unscheduled, as required.)

1-14

d. PROGRESS MEETINGS.

Monthly meeting with Contracting staff and Veterans.

3 & 6

8. RATINGS

Metrics and methods are designed to determine if performance exceeds, meets, or does not meet a given standard and acceptable quality level. A rating scale shall be used to determine a positive, neutral, or negative outcome. The following ratings shall be used:

Quarterly Reports will be used to reflect the rating of “exceeds, meets, or does not meet a given standard. These reports will be combined to incorporate into the annual inspection assessment.

9. DOCUMENTING PERFORMANCE

a. ACCEPTABLE PERFORMANCE

The Government shall document positive performance. Any report may become a part of the supporting documentation for any contractual action.

b. UNACCEPTABLE PERFORMANCE

When unacceptable performance occurs, the COR shall inform the contractor. This will normally be in writing unless circumstances necessitate verbal communication. In any case the COR shall document the discussion and place it in the COR file.

When the COR determines formal written communication is required, the COR shall prepare a Contract Discrepancy Report (CDR), and present it to the contractor's program manager.

The contractor shall acknowledge receipt of the CDR in writing. The CDR will specify if the contractor is required to prepare a corrective action plan to document how the contractor shall correct the unacceptable performance and avoid a recurrence. The CDR will also state how long after receipt the contractor has to present this corrective action plan to the COR. The Government shall review the contractor's corrective action plan to determine acceptability.

Any CDRs may become a part of the supporting documentation for any contractual action deemed necessary by the CO.

10. FREQUENCY OF MEASUREMENT

a. Frequency of Measurement.

During contract performance, the COR will periodically analyze whether the negotiated frequency of surveillance is appropriate for the work being performed.

b. Frequency of Performance Assessment Meetings.

The COR shall meet with the contractor as needed to assess performance and shall provide a written assessment.

**DESMONA
JOHNSON**

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JOHNSON
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Signature –Program Manager

PAUL SCHEPPF

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Date: 2025.11.21 14:50:28 -06'00'

Signature – Contracting Officer's Representative

Signature – Contracting Officer